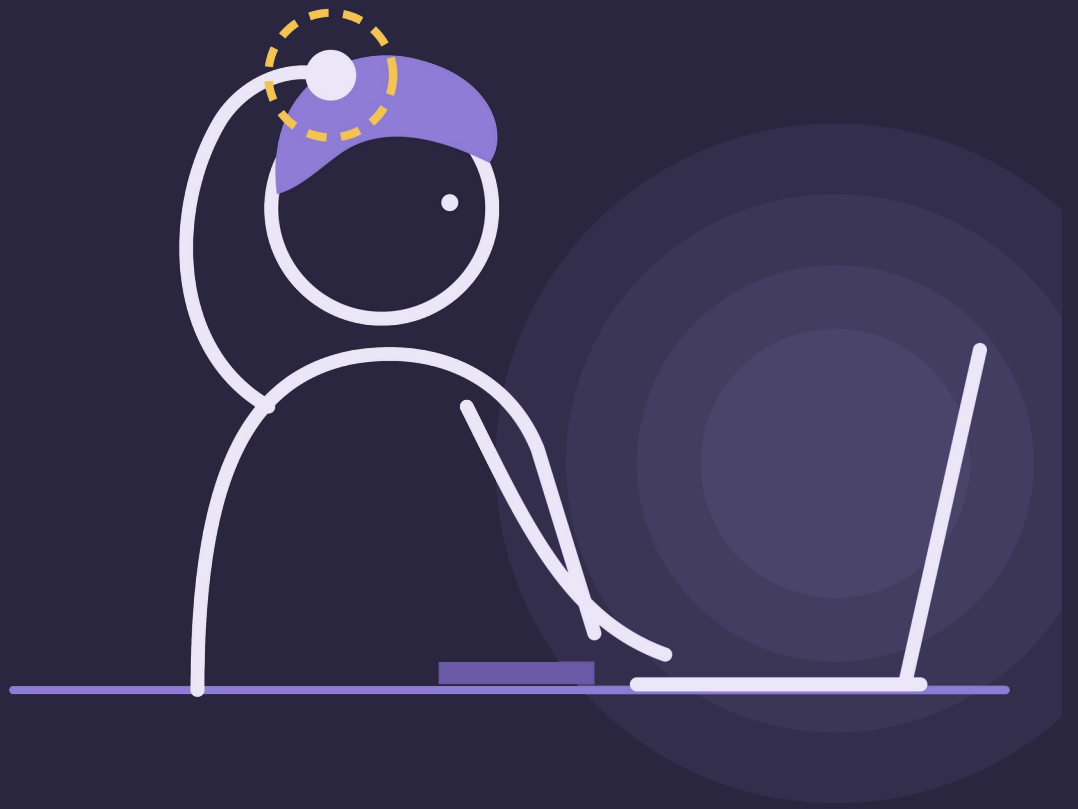
11:42 pm

It starts before you notice

Studying. Reading. Working. And somewhere in the middle, a hand drifts to the hair.



TrichFree: when your hand moves, support responds.

Help arrives after the moment has passed

1.7%

of adults live with trichotillomania: about 1 in 60

17.7

mean age it begins, right at university age

1 in 4

report daily interference with their work

> *If you could get each individual to record something for themselves which would help them at the moment of the behaviour and somehow AUTOMATICALLY play it back to them when they began pulling, I think it would stop them.*

Practitioner with 30+ years of experience supporting people with trichotillomania

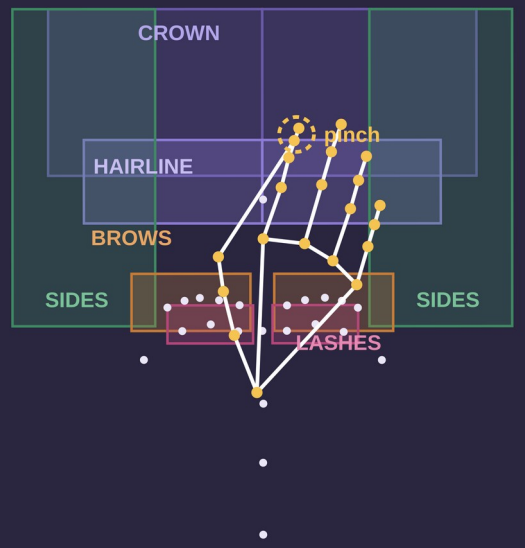
Sources: Grant, Dougherty & Chamberlain, Psychiatry Res. 2020 (n = 10,169); Woods et al., Trichotillomania Impact Project, J Clin Psychiatry 2006.

Habit-reversal therapy works, and its first step is awareness. But automatic pulling skips that step, and therapy is weekly while urges are not.

Meet TrichFree

A browser extension that uses the webcam you already have. When your hand moves, support responds.

WHAT TRICHFREE SEES



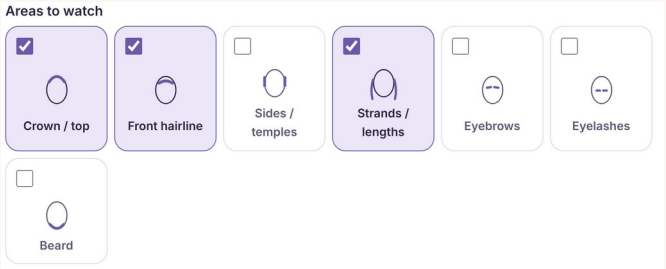


Hair mask from the hair-segmentation model on our demo video

21 hand points · face reference frame · hair segmentation
Zones are measured in face units, so they follow your head as you move. Shown: a synthetic test recording.

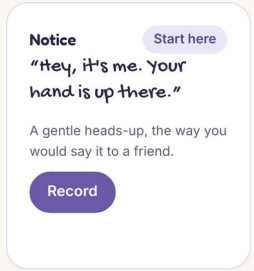
1 Notice

Finds a pinch in the areas you choose, including a lock held beside the face. A 0.8–1.5 s pause keeps brief touches quiet.



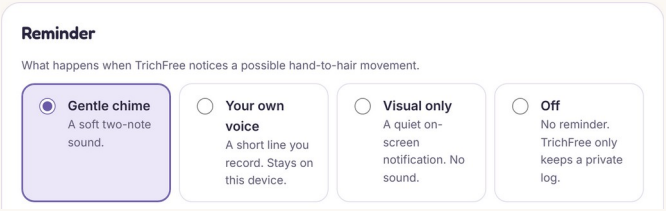
2 Hear yourself

A soft chime, then a line you recorded: “Hey, it’s me. Your hand is up there.”



3 Stay in control

Your voice, a chime, a visual cue or off. Video never leaves the laptop; voice clips are never uploaded.



Competitive Positioning

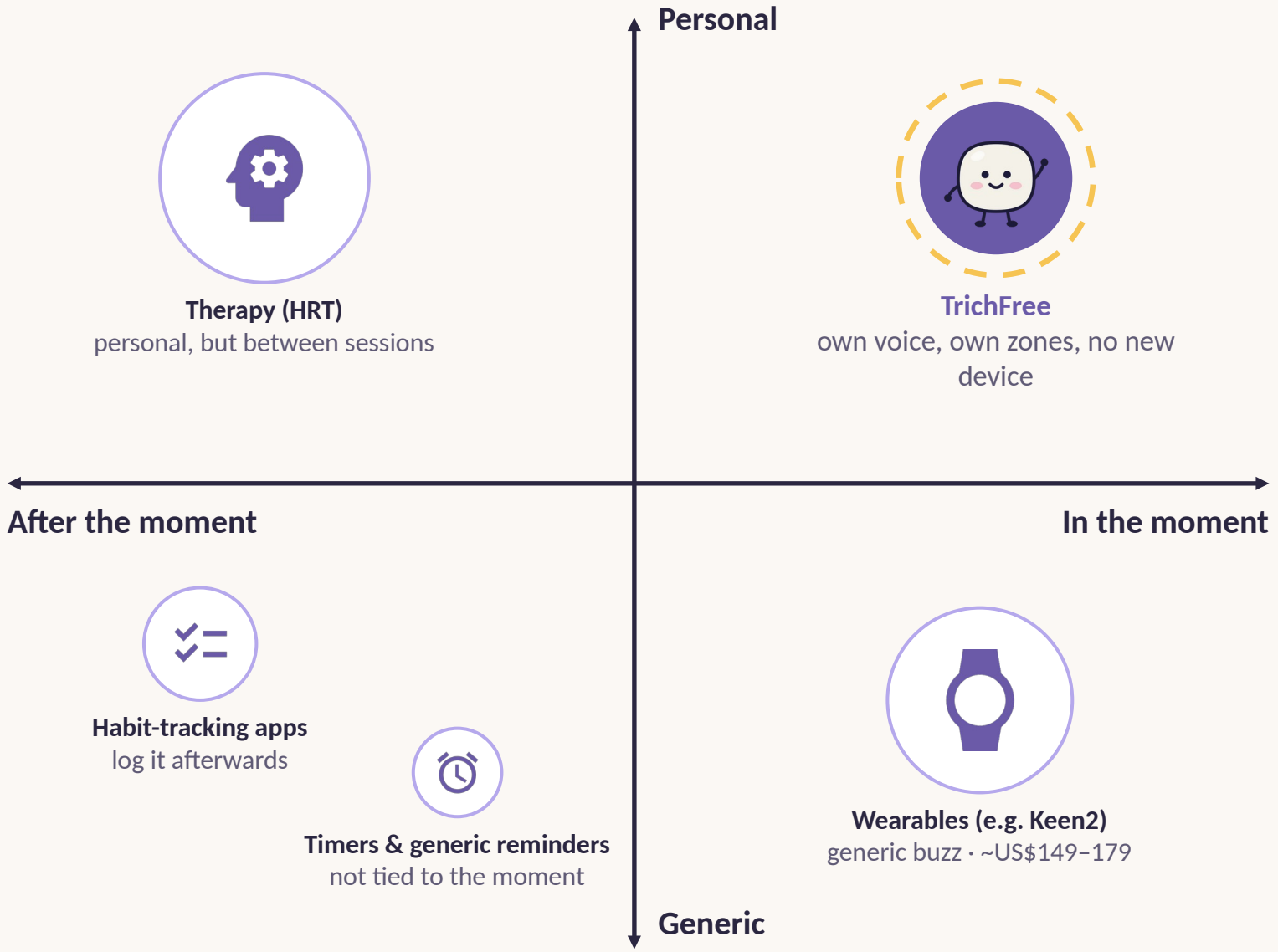
Where support meets the moment

TrichFree is the only option that is both **in the moment** and **personal**, on the webcam students already own.

Bubble size = cost to the user

Low

High



Positions are our assessment. Therapy stays the gold standard, and TrichFree complements it. Wearables go everywhere; we only see the desk.

Built, tested, and ready to be proven

Working extension + website

Chrome/Edge extension, companion site and an in-browser demo.

283 automated tests

192 unit · 57 database · 34 browser. Software reliability, not detection accuracy.

350 / 350 simulated reaches

Caught by our evaluation harness, with 0 false alerts on everyday moves. Synthetic data.

Hair-aware detection

Added hair segmentation after our first user test missed a strand pull.

Private by design

Video never stored or sent. Our own release audit found and fixed 11 issues.

Expert engagement

A practitioner with 30+ years of experience offered to stay in touch and help.

NEXT Label our 100+ founder trials → measure real-world accuracy with new users → 14-day pilot (already built in)

Support should not be a privilege

WHO WE CAN REACH

~1 in 60 adults
live with trichotillomania worldwide

~80,000 in Singapore
adults, estimated from 1.7%

~680 at NUS
students: our beachhead, estimated

Why the economics work
All AI runs on the user's laptop: no cloud GPUs, no video storage. Each extra user costs almost nothing, so a free tier is sustainable.

Estimates apply 1.7% (Grant et al. 2020) to Singapore's adult population and ~40,000 NUS students. Illustrative only.

Individuals

Free

Core detection and own-voice reminders. No device to buy: a wearable costs ~US$150+, a webcam is already there.

Institutions

Paid licence · hypothesis

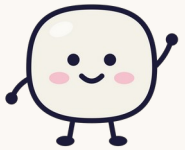
University wellbeing services and BFRB organisations offer supported access to their communities. Willingness to pay is untested: we test it in Stage 3.

Practitioners

Long-term · not built

A between-session tool clients choose to use, with consent-based sharing. The same hand-to-face engine could later support other BFRBs.

Every step earns the next



STAGE 1 · Nov–Dec 2026

Validate

Label our 100+ trials; observe 3–5 first-time users

NUSC: Technical & research mentors

Measure: *Sensitivity · false alerts per hour*

STAGE 2 · Q1 2027

Pilot

Ethics-reviewed 14-day pilot: 3-day baseline, then chime vs own voice

NUSC: Ethics guidance · participant costs

Measure: *Helpfulness · acceptability · hand-down rate*

STAGE 3 · Q2 2027

Distribute

Trials with NUS wellbeing services and BFRB communities

NUSC: Introductions · business mentorship

Measure: *14-day retention · partner interest*



STAGE 4 · H2 2027 →

Personalise

Practitioner-informed features; consent-based sharing

NUSC: Clinical mentorship · funding

Measure: *Partner uptake · zero privacy incidents*

Engagement is not clinical effectiveness: we measure them separately. Dates are targets.

Let’s ease suffering, one quiet moment at a time

photo

[Founder 1 name]
Robotics & engineering: built the computer-vision pipeline

photo

[Founder 2 name]
Statistics: designed the evaluation and randomised pilot

We kept building through hackathon setbacks. When our first user test failed, we fixed it within days.

Why incubation, not just more time

We can build the software. We can’t yet run an ethical study, reach institutional buyers or form clinical partnerships alone.

- Clinical & research mentorship
- Ethics-reviewed pilot design
- Access to student users
- Business-model & willingness-to-pay testing
- Introductions to wellbeing & BFRB organisations

Tonight at 11:42, someone’s hand will drift to their hair. We want the next thing they hear to be their own voice: “It’s okay. I’ve got you.”

